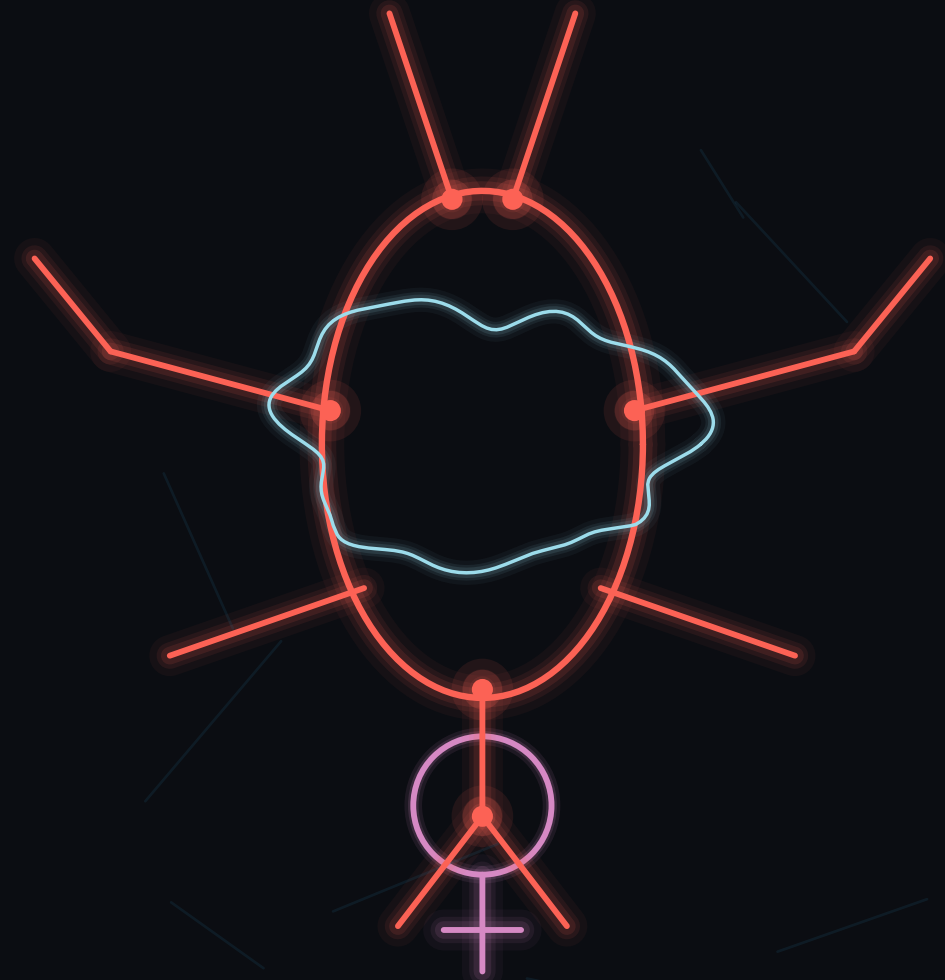


The Center for **Women's Neurovascular Disease**

*Closing the sex gap in how we prevent, diagnose
and treat stroke across a woman's life.*



Stroke is not sex-neutral.

Yet the way we predict it, recognise it and treat it was built around the average patient — and the average patient is a man.

3×

migraine — a stroke risk multiplier — in women

7×

lupus, a vascular brain disease, in women

78%

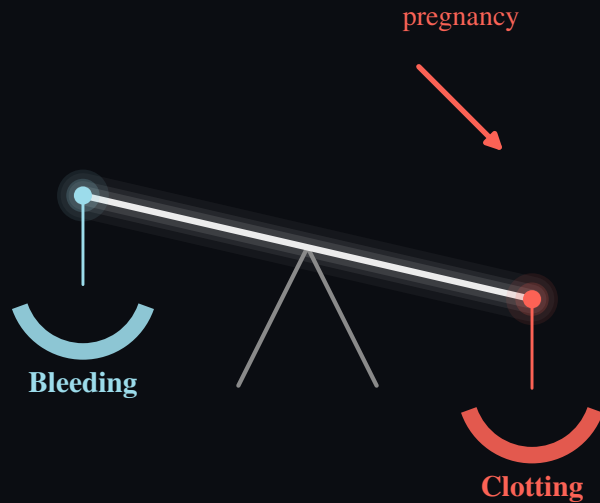
of autoimmune disease is in women

>

atrial fibrillation hits women harder than men

A whole population whose cerebrovascular risk runs on a different set of rules.

The same vessels, a different operating system



Pregnancy re-tunes hemostasis toward clotting.



Hormones set the dial

Estrogen tunes vascular tone, clotting and migraine — and shifts the stroke risk with it.



Pregnancy is a vascular event

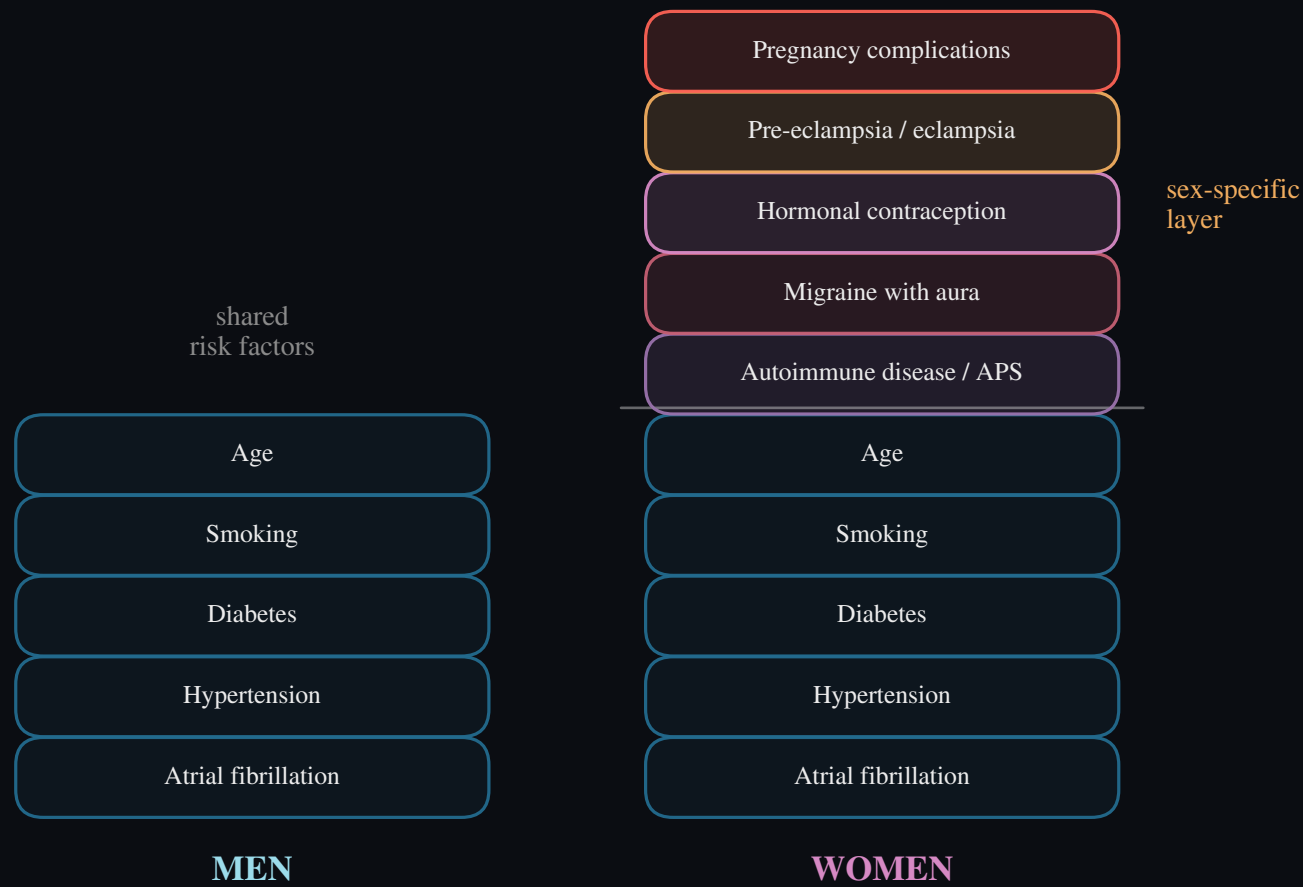
A months-long pro-thrombotic, high-pressure state every man is spared.



Autoimmunity targets vessels

Female-predominant disease (lupus, APS) attacks the brain through its blood supply.

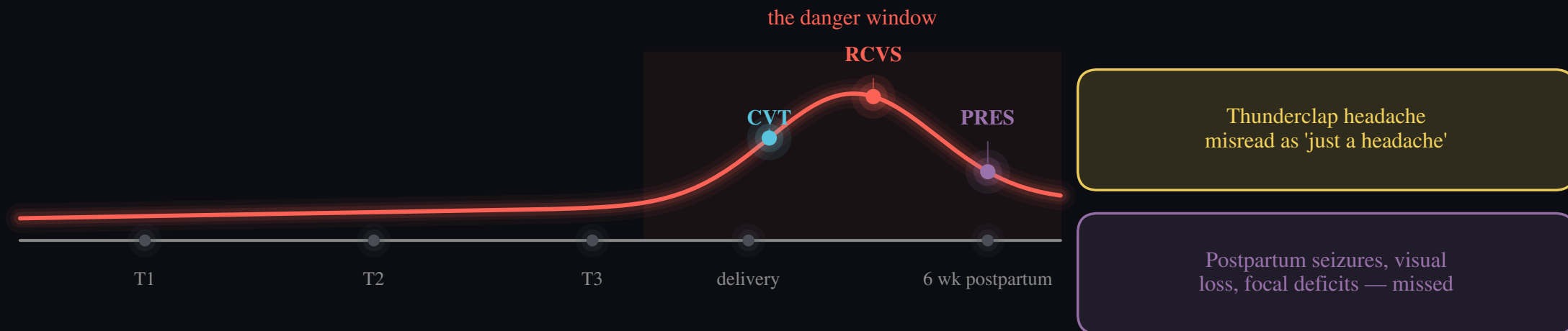
Every risk factor men have — plus a layer of their own



Women carry the entire male risk profile — **and then a second one no man ever faces.**

Even shared factors aren't equal: in atrial fibrillation, women's stroke risk is **6.2% vs 4.2% in men.**

A cluster of strokes hiding behind a headache



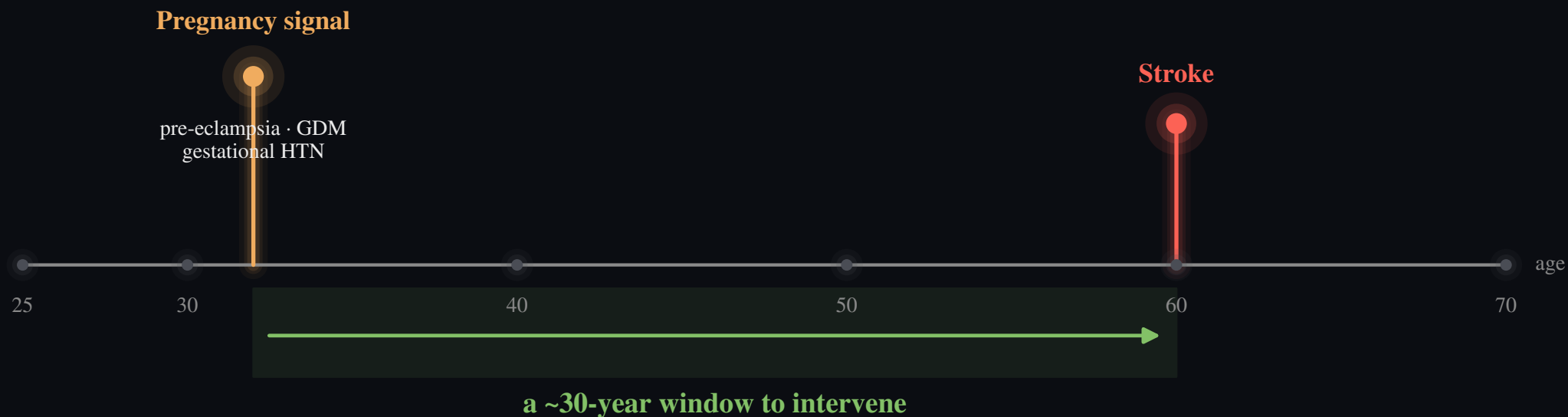
PRES, RCVS and cerebral venous thrombosis overlap, peak in the same weeks,
and present through symptoms that are easy to dismiss — when minutes matter.

Pregnancy is a stress test we forget to read



These outcomes forecast cerebrovascular disease decades early — yet a **reproductive history rarely makes it into a stroke-risk assessment.**

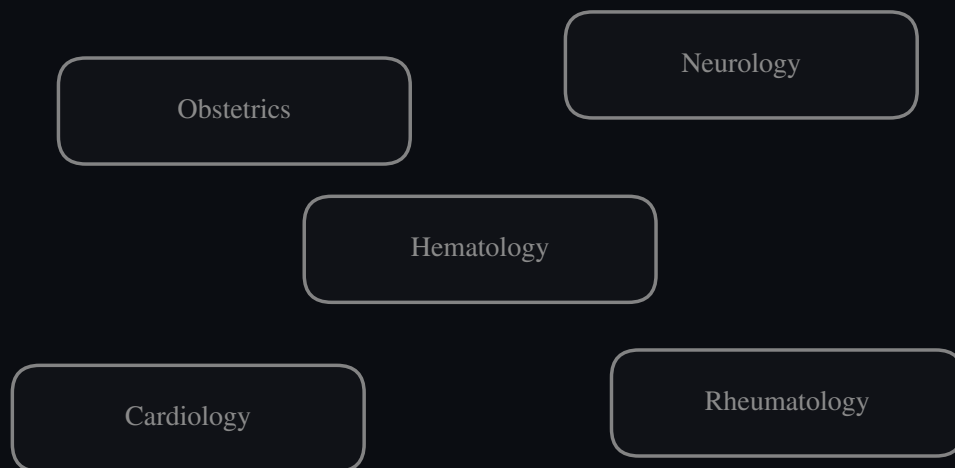
The earliest warning in medicine — currently ignored



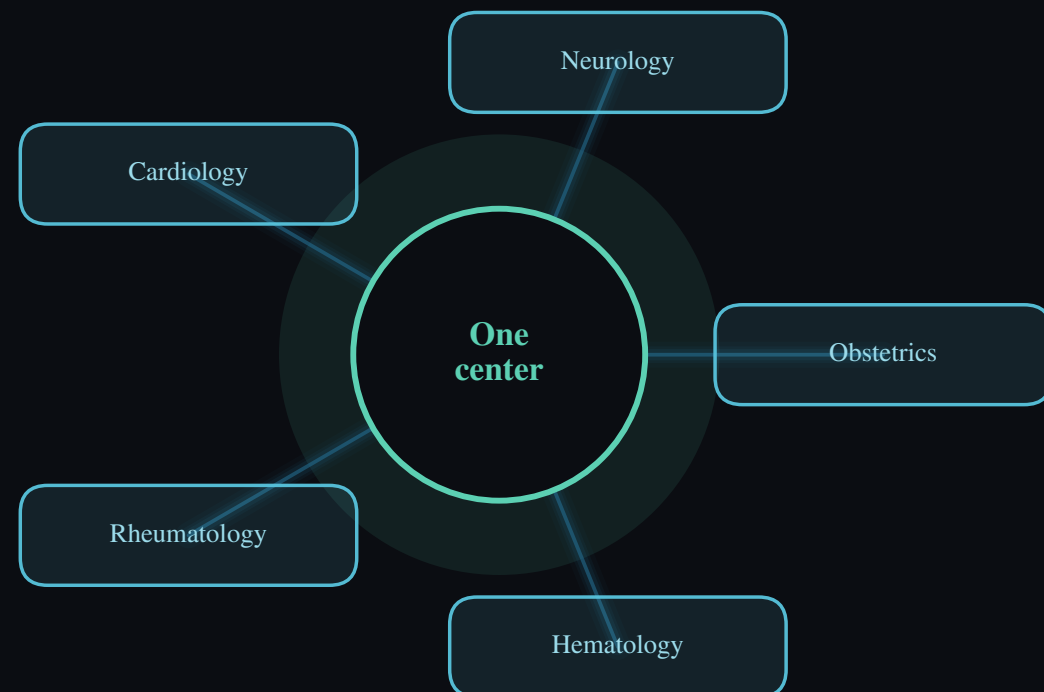
Most prevention begins far too late. The female-specific signal arrives decades earlier — if someone is set up to act on it.

No one owns the whole woman

Today: fragmented



Each sees a slice.
The woman falls through the gaps.



One team, one record,
one woman — across her life.

What the center will do

Peripartum stroke care

Rapid recognition and treatment of PRES, RCVS and venous thrombosis.

Lifelong risk stratification

Turn every reproductive history into an early stroke-prevention plan.

Sex-specific research

Study the mechanisms and trials that the average-patient model ignored.

Education & advocacy

Train clinicians to see the risk — and the woman — that others overlook.

Half the population. A different disease. A center built for it.

Data: O'Neal MA. A Review of Women's Neurology. Am J Med. 2018;131(7):735–744. · doi.org/10.1016/j.amjmed.2017.11.053